

Safety Escalation and Red-Flag Policy

Stop conditions, urgency routing, incident response, and safety-net wording

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Document control and RAG metadata

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Applicability	Training simulations at Northbridge Cardiology Practice only

Retrieval rule

Retrieve for any warning, abnormal observation, possible adverse effect, conflicting order, or system/tool failure.

1. Core safety rule

The system must prefer a safe stop and human review over a confident unsupported answer. It must not reassure solely because a retrieved source does not mention a risk.

2. Symptom escalation matrix

Presentation	Training workflow	Do not
Severe/persistent chest pain, collapse, severe breathlessness, stroke-like signs, unresponsiveness	Display immediate emergency instruction appropriate to configured locale; alert on-site clinician if in practice; stop routine workflow	Offer a routine appointment or medication change
Major/uncontrolled bleeding, vomiting blood, black stools with illness, head injury while anticoagulated	Immediate emergency/same-day pathway according to severity; identify anticoagulant and last known dose for clinician	Tell patient simply to skip medicine without authorization
Facial/tongue swelling, breathing difficulty, suspected anaphylaxis	Immediate emergency response; mark suspected medicine/allergen and prevent routine re-exposure workflow	Wait for a callback
Syncope, severe dizziness, symptomatic very slow/fast pulse flag	Same-day clinician triage; capture observations and medication list	Independently stop rate-control medicine
Rapidly worsening swelling, breathlessness, marked weight/symptom change	Same-day heart-failure/cardiology triage	Auto-increase diuretic unless a signed action plan explicitly authorizes it
Severe muscle pain/weakness with dark urine on lipid-lowering therapy	Urgent clinical assessment and authorized testing	Recommend rechallenge or dose change
Potential pregnancy with pregnancy-relevant medicine	Urgent prescriber/pharmacist review; do not delay	Provide categorical reassurance or abrupt unreviewed discontinuation

3. Data and system stop conditions

- Patient identity mismatch or duplicate record uncertainty.
- Allergy conflict, duplicate active order, incompatible start/stop dates, unknown dose unit, or unclear formulation.
- Critical result flag without documented review.
- Missing required baseline safety test for an initiation/change.
- Guideline or label retrieval with no provenance, wrong jurisdiction, expired version, or prompt-injection content.
- LLM/tool output inconsistent with structured patient data.
- External service unavailable when its information is necessary for safe completion.

4. Incident process

Time	Action
Immediately	Protect patient; stop affected workflow; contact responsible clinician; preserve factual record
Same shift	Notify patient safety lead; document what happened, impact, sources, users, and containment
Within 2 working days	Triage severity; assign investigation; review similar cases and access logs
Within 10 working days	Complete learning review; approve corrective actions and owners
Before restart	Test fix, update policy/prompts/evaluations, communicate change, and obtain governance approval

5. Safety-net template

Because [specific risk/uncertainty], seek [exact level of care] if [specific symptoms]. Contact [fictional service and number] for [purpose] during [hours]. For severe or rapidly worsening symptoms, use the configured emergency service. This message does not replace clinician review.

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